Epithalon (Epitalon) – Anti-Aging Peptide

Step-by-Step Reconstitution Process

- **Gather supplies: BAC water, alcohol swabs, insulin syringe, vial**
- Bring BAC water and peptide vial to room temperature before mixing
- **Removing plastic cap.** Carefully pop off the plastic cap, do not remove metal flange.
- **Clean both vials:** Wipe the rubber stoppers with alcohol swabs
- **Draw BAC water:** Pull your chosen amount into the syringe
- **Inject slowly:** Add the water to the peptide vial, aiming it at the glass wall—not directly onto the powder
- **Swirl gently:** Roll the vial between your palms. Never shake.
- **Wait:** Let it sit 5-10 minutes. Solution should be crystal clear.
- **Store:** Refrigerate immediately. Good for 28-30 days once reconstituted.

If it's cloudy or won't dissolve: Let it sit longer. Try gentle warming (under your arm or in lukewarm water). If still cloudy after 30 minutes, contact your source.

Practical Protocol

Based on the available research comparing crude extract to synthetic compound:

STANDARD PROTOCOL

Dose	500 mcg to 1 mg per day
Frequency	Once daily
Timing	Evening (aligns with melatonin mechanism)
Administration	Subcutaneous injection
Duration	10 to 20 consecutive days per cycle
Cycles	2 to 3 times per year with several months between

RECONSTITUTION (10 mg vial)

Bac water	2 mL
Concentration	500 mcg per 10 units on insulin syringe 1 mg per 20 units on insulin syringe

Research on continuous dosing showed no additional benefit over periodic cycles. Think of it like hitting a reset button rather than maintaining constant levels. You run a short cycle to activate these processes, let your body maintain that activation on its own, then come back a few months later to reinforce it.

Stacking Context

Epithalon + SS-31 + FOXO4-DRI (Longevity Stack for 50+)

For adults over 50, this combination functions as a cleanup crew. FOXO4-DRI eliminates senescent cells to reduce inflammation. Epithalon repairs telomeres on cells that have split too many times so they have a longer life. SS-31 repairs mitochondria. These can all be run together. After the SS-31 phase is complete, you can transition to MOTS-c and NAD+ for optimization.

Epithalon + NAD+

This combination targets multiple aspects of cellular aging. Epithalon supports telomere maintenance while NAD+ supports mitochondrial function and DNA repair. Different mechanisms, complementary effects.

Epithalon + Growth Hormone Peptides

No interaction concerns. Different timing requirements. GH peptides require fasting. Epithalon does not. Inject Epithalon in the evening and keep GH peptides on their own fasting schedule.

Epithalon + GLP-1 Agonists

No interaction concerns. Completely different mechanisms. Can be run concurrently without timing conflicts.

Common Questions

How is Epithalon different from taking melatonin supplements? Melatonin supplements provide exogenous melatonin that your body uses and clears. Epithalon stimulates your pineal gland to produce its own melatonin again, restoring your natural production. That means the effects persist after you stop taking Epithalon because your gland has been reactivated, whereas melatonin supplements only work while you are taking them.

Why such short cycles? Epithalon activates and resets cellular processes. Once telomerase is reactivated and melatonin production is restored, those effects persist for months. Continuous dosing showed no additional benefit over periodic cycles in the research.

Will I feel the telomere effects? No. Telomere lengthening is not something you perceive. The benefits are cellular and play out over years in terms of reduced disease risk and maintained cellular function. What you will feel is the melatonin and sleep improvements.

Is this safe with cancer or cancer history? The relationship between telomerase activation and cancer is complex. Cancer cells often have high telomerase activity. However, a 2025 cell study showed Epithalon uses a different mechanism in cancer cells (ALT pathway) than in normal cells (telomerase), and animal data consistently showed reduced rather than increased cancer risk. Nonetheless, people with active cancer should avoid Epithalon until more is known. Talk to your oncologist first.

Why are nearly all the studies from Russia? Professor Khavinson's lab discovered Epithalon and has conducted the research program since the 1980s. No Western research group has independently replicated the findings. This does not invalidate the work, but it means the evidence base relies

on a single research group. The mechanism makes biological sense and the animal data is consistent, but independent confirmation would strengthen the case significantly.

Side Effects

In Published Research

Epithalon has been studied for over 35 years with a strong safety record. The 162-patient retinitis pigmentosa study reported zero side effects. The 12-year cardiovascular study reported no severe adverse events. A 2025 review noted that formal safety data remains limited, stating that "information regarding critical issues about this peptide's safety is missing."

What Users Report

Common (mild): Injection site reactions (redness, swelling, mild discomfort), vivid dreams, temporary changes in sleep patterns during the first few days

Uncommon: Mild headache, mild fatigue during initial use, drowsiness (especially early in a cycle)

Most reported side effects are mild and temporary.

Contraindications and Precautions

Do Not Use If You Have:

- Active cancer or tumors (telomerase activation raises unresolved questions in humans)
- History of malignancy (consult your oncologist before use)
- Known hypersensitivity to Epithalon or any component

Use Caution With:

- Pregnancy or breastfeeding (no safety data)
- Autoimmune conditions
- Any condition requiring medical supervision

Drug Interactions:

- No well-established drug interactions due to limited research
- If you are taking exogenous melatonin, be aware that Epithalon will also increase your endogenous melatonin production, potentially compounding effects